



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رباب احمد رمضان محمد
المحافظة : القاهرة
الحى / المدينة : المقطم
رقم البطاقة : 27909090100785 رقم التليفون : 01155030939
العنوان : ممممممممم
العمر (تاريخ الميلاد) : 09-09-1979 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 19-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رباب احمد رمضان محمد National ID: 27909090100785	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
وزارة الداخلية
شهادة ميلاد

رئيس
أحمد رمضان محمد
١٤٤ مسكن صبيح حسن
القطر - القاهرة

٢٧٩.٩.٩.٠١.٠٠٧٨٥

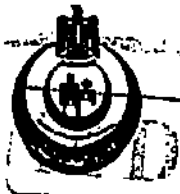
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٢٠٣٠/٠٧
٢٧٩.٩.٩.٠١.٠٠٧٨٥
حاصلة على بطون مسجل

إثني
محمود محمد محمد حسن
البطانة ساريه حسن

٢٠٣٧/٠٧/٢٤

١



مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

السلطة المصرية
صحة
مصر

MDT FORM

17/12/2024 GM

Name	رانيا ام رمضان	MRN	٢٠٢٠-٤٩
Age	45 yrs.	ID	٢٧٩٠٩٠٩٠١٠٠٧٨٠
Menopausal status		Phone	٠١١٠٢٢٩٧٩٩٩
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	Rem DT

Clinical Data

BX:

ER:

8/8

PR:

3/8

HER 2:

-ve

Ki67:

R

L

Sonomammography:

2019 → Rt BCS T2N2 → ACX4 → T9a2l

Mets Workup:

Zoledex + tam 6/2021 → 10/2024 TNM, bone, L.N

PET-CT = 12/2024: bone mets, Rt supraclav. L.N.
2 Rt axillary L.N. & Hf1s

Recommendations:

MDT OUTCOME & PLAN

For Zoledex Femara / Ibuprofen
Remetix 3 months, palliative Rx to bone

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric



Patient Name: Rabab Ahmed Ramadan Mohamed	Date : 05/02/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 45 Year(s)
Medical ID: 49/2020	Pat. ID: 15706

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- Bilobar hypoechoic focal lesions, the largest is subcapsular at segment VII measuring 38x23mm
- No intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Surgically removed
- CBD is not dilated.

Spleen: -

Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Bright liver likely fatty with bilobar focal lesions likely metastases

Cholecystectomy

Signature Dr . Nermien Ramadan

05/02/2025 2:06 PM



Patient name: Rabab Ahemd

Date: 25-11-2020

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

- Follow up post right lumpectomy.
- No available prior studies for comparison.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR A).
- Right breast UOQ operative bed asymmetric density with related parenchymal distortion, corresponding by U/S to (8-9 o'clock) surgical thick scarring tissue with related ill-defined area of heterogenous parenchymal echogenicity. No internal vascularity on color Doppler application.
- Interstitial edematous changes of the right breast.
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- No left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- No pathological right axillary LNs.
- Free right infra and supra clavicular regions.

Opinion:

- Follow up post right CBS showing marked operative bed scarring with related area of parenchymal distortion and possible area of fat necrosis (BIRADS III). Further assessment by CE-MRI is advisable.
- Normal study of the left breast (BIRADS 1)

Yours truly

Dr. Peter Fahmy



Patient Name: Rabab Ahmed Ramadan Mohamed	Date : 10/10/2024
Exam.: C.T CHEST	Age : 45 Year(s)
Medical ID: 49/2020	Pat. ID: 15706

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Known case of breast cancer.

1. Chest:

Comparing to the previous study dated 16/01/2024

- **Still noted:**
 - Right breast UOQ parenchymal distortion owing to prior surgery.
 - Single right upper lobe subcentimetric pulmonary nodule is seen measuring about 3 mm in diameter.
 - Right sided middle lobar sub-pleural reticulations.
- No sizable left pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

Comparing to the previous study dated 16/02/2023

- The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic collections.

3. Skeletal scan show:

- **Newly developed:**
 - L3 vertebral body, right ischium and right humeral tiny sclerotic bone lesions.
 - Left iliac faint lytic lesion is seen breaching the cortical line as well as D6 vertebral body lytic bone lesion.
- **Still noted** bilateral iliac, right humeral head and right scapular sclerotic foci.



Patient Name: Rabab Ahmed Ramadan Mohamed	Date : 10/10/2024
Exam : C.T CHEST	Age : 45 Year(s)
Medical ID: 49/2020	Pat ID: 15706

OPINION:

- ***Newly developed L3 vertebral body, right ischium and right humeral tiny sclerotic bone lesions as well as left iliac and D6 vertebral body lytic bone lesions, for bone scan correlation.***
- ***Otherwise, stationary course of the rest of today's study.***

Dr. Maitha Fadhl
Dr. Reham Anwar, FEBR, MD



مؤسسة مستشفى سرطان
الأطفال - مصر

Nuclear Medicine Dept.
PET/CT Unit

Name: Rabab Ahmed Ramadan Mohamed
Referred by: Prof. Mohamed Abd Elmaksoud

Date: 4/12/2024
MRN: 30188888

F18-FDG PET/CT STUDY

CLINICAL HISTORY:

- Female patient with history of treated right breast cancer.

REASON FOR PET/CT STUDY:

- Restaging.

TECHNIQUE:

- Radiopharmaceutical: 18F FDG, MBq intravenously.
- Blood glucose at time of 18F-FDG injection: - mg/dL.
- Time from 18F-FDG injection to scan: 60 min
- PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- Additional notes: SUV max normalized to body weight was used.

FINDINGS:

Head and Neck:

- Few FDG avid right supraclavicular lymph nodes the largest measures 1.6 cm with SUV_{max} ~7.7
- NO FDG avid thyroid nodules.



1 Seket Al-Elmam st, El Sayda Zainab, Cairo

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E-mail: info@57357.org

Knowledge into Action

www.57357.org

1 شارع سكة الإمام - السيدة زينب - القاهرة

ص. ب: ١ الدواوين - مصر

الهاتف: (202) 19057

فاكس: (202) 25 35 1745

57357Egypt

PET Scan

[[DOCTYPE:PET_SCAN]]



- * Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- * Normal diffuse brain activity with NO hypermetabolic brain lesions.

Chest:

- * Status post right breast conservative surgery showing: right breast operative bed ill-defined soft tissue densities and sheets with central fat density with mild FDG activity of SUVmax~2.3, seen inseparable from the underlying pectoral muscle ?? post-operative changes with fat necrosis, for clinical correlation and further mammography and US assessment
- * Right axillary lymph node measuring 1.3 cm showing mild FDG activity of SUVmax~2.8.
- * No left breast FDG avid mass
- * There is no hypermetabolic mediastinal or left axillary lymphadenopathy.
- * Right middle lung lobe subpleural fine reticulations with overlying minimal pleural thickening, likely sequel of radiotherapy
- * No FDG avid lung nodules or masses detected.
- * No metabolically active pericardial or left pleural lesions detected.
- * Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- * Mildly enlarged fatty liver showing few FDG avid focal lesions the largest at segment VI measuring about 3.4 cm with SUVmax~5.1
- * Few portahepatis and portocaval lymph nodes the largest measures 1.6 cm with mild FDG activity of SUVmax~2.2
- * There is no hypermetabolic pelvic lymphadenopathy.
- * The spleen is of average size with NO FDG avid focal lesions detected.
- * The pancreas shows normal FDG avidity.
- * There are no active adrenal nodules.
- * Underfilled urinary bladder showing mild circumferential mural thickening, likely cystitis





- * Physiologic FDG excretion is seen in the kidneys and bladder.

Musculoskeletal:

- * Multiple FDG avid bone lesions are seen involving multiple vertebrae, pelvic bones, left femur, right scapula and left 1st rib showing variable grades of FDG activity the highest recorded at the right iliac bone of SUVmax~15.1.

IMPRESSION:

FDG PET/CT study for a known patient of treated breast cancer, revealed:

- * Status post right breast conservative surgery showing: right breast operative bed ill-defined soft tissue densities and sheets with mild FDG activity ?? post-operative changes with fat necrosis, for clinical correlation and further mammography and US assessment
- * FDG avid right supraclavicular, right axillary and upper abdominal nodal lesions.
- * FDG avid metastatic hepatic focal lesion (deposit).
- * Multiple FDG avid metastatic osseous deposits.

Thanks for referral;

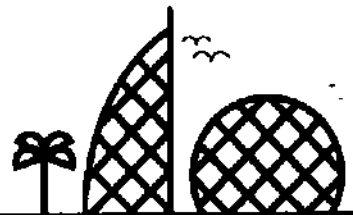
Dr Abeer Ali, MD.

Dr Esraa El-Khoily, MD

Consultant of Radio-diagnosis.

Consultant of Nuclear Medicine, CCHE
Associated Professor of Nuclear Medicine, NCI

Cairo University



Name : Rabab Ahmed Ramadan Mohammed

Date : 23-04-2026 11:16

Branch : Dokki

Medical No. : 358100

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعافد مستشفيات

Tc-99m MDP ISOTOPIC BONE SCAN

Clinical History: Female, 47 years old; History of breast carcinoma, complains of bone aches. Evaluation for possible skeletal metastases.

Technique: Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time.

Findings:

- o **Right acetabulum:** Small, faint focal uptake; Subtle finding—cannot definitively differentiate early metastasis from degenerative change (hip osteoarthritis), for clinical and radiological correlation.
- o Mild, heterogeneous activity throughout the cervico-dorso-lumbar spine; no discrete focal lesions identified.
- o Bilateral increased uptake in shoulders, knees, and ankles, consistent with multi-joint degenerative changes.

CLINICAL IMPRESSION:

- o Equivocal right acetabular uptake: Degenerative arthropathy more likely, but early metastasis not entirely excluded, for radiological correlation.
- o No other foci suspicious for active metastatic disease.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



Patient's Name: RABAB AHMED RAMADAN
Date: 2026-03-30
Referred by: Prof.Dr. BRAIN+C
Hospital: Goustave Roussey International Egypt

MRI of the brain using different pulse sequances& parameters revealed:

Sequances;Axial T1,T2&FLAIR, DWI , Sagital7 Coronal T2, T1 post contrast

Findings:

No areas or foci of diffusion restrictions seen

-Normal appearance of cerebral ventricles with no smidline shift or deformity seen.

-No definite parenchymal areas of abnormal signal seen.

-Normal cortical sulci & cerebral fisuures

Normal posterior fossa.

Normal sellar region.

Right maxillary small retention cysts

CONCLUSION:

Essentially Normal MRI brain study

Dr.Abubakr Hejazi MSc FEBR

Patient's Name: **Rabab ahmed ramdan mohamed**
Date: **2026-04-30**
Referred by: **Prof.Dr. cap+c**
Hospital: **Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Compared to the previous study dated 29/1/2026, the current study showed:

I. CHEST:

- Right apical subpleural reticulations likely post therapeutic sequel.
- Linear atelectatic bands are noted
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II. ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

III examined skeleton

Multilevel vertebral bodies, bilateral iliac bones, sacral& sternal sclerotic lesions

Opinion:

- **Over all stationary course**

• **DR/NERMEEN RAMADAN.MD**

Dr Nermin Ramadan

Dr Nermin Ramadan

معمل حلوان للتحاليل الباثولوجية و الانسجة

أ.د. سحر محمد طلعت
دكتوراه واستشاري الباثولوجيا
أستاذ الباثولوجيا كلية الطب جامعة القاهرة

أ.د. سامية محمد جيل
دكتوراه واستشاري الباثولوجيا
أستاذ الباثولوجيا كلية الطب جامعة القاهرة

PATHOLOGY REPORT

Name: رباب أحمد رمضان

No: 869/2019

Age: 40 Y

Referred By: Prof. Dr. Abdelmaksoud Mohammed.

Specimen: 1) Breast mass & sentinal LN. for frozen. 2) Axilla. Date: 26/12/2019

GROSS:

- 1) Breast mass & sentinal LN. for frozen: An irregular piece of fibrofatty tissue measured 6x5x5 cm with designated margins. Cut section revealed an ill defined mass 2.8x2.5x2 cm with least margin was 0.5 cm. Also received one lymph node 1.5 x1 cm with firm consistency and whitish cut section.
- 2) Axilla: Fibrofatty tissue pieces 8x8 cm collectively entangling 21 lymph nodes the largest 2x1.5 cm.

MICROSCOPIC:

Microscopic examination of sections prepared from the mass detected revealed malignant tumor tissue that is formed of infiltrating sheets and files of malignant ductal cells, with moderate degree of anaplasia. The tumor cells are surrounded by moderate desmoplasia and intense inflammatory infiltration. Foci of lobular cancerization are seen. The resection margins are free from tumor. Examination of sections prepared from the sentinal lymph node showed metastatic deposits and examination of the received axillary lymph nodes revealed metastatic deposits in five of the dissected twenty one lymph nodes.

DIAGNOSIS:

- 1) BREAST MASS FOR FROZEN. 2) AXILLA:
- INFILTRATING DUCT CARCINOMA, GRADE II.
- RESECTION MARGINS ARE FREE FROM TUMOR.
- METASTATIC DEPOSITS IN AXILLARY NODES (6/22). (T2 N2 Mx).

Sixteen slides enclosed.

Prof. Dr. Samia M. Gabal
Samia Gabal

Patient Name : الأستاذ / رباب احمد رمضان	Request No. : 13004179
Patient No. : 49-2020	Request Date : 15/01/2025
Path No. : 175/25	Verification Date : 05/02/2025
Sex : Female	Financial Category : نفقة التولية - اورام دار السلام
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Cervical lymphadenopathy, with suspicious right supraclavicular lymph node.
Nature of Specimen: Tru-cut biopsy, collected at 14/01/2025.

Gross Examination:

- Three grayish white tissue cores up to 0.5 cm., totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Not detected.
- * Perineural invasion: Not detected.
- * Angiolysis: Not detected.
- * Microcalcifications: Not detected.
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in 7% of tumor cells (score 3/8).
- Her2: No membrane reaction of tumor cells (score 0).
- Ki67: 20% nuclear reaction of tumor cells.

Diagnosis:

Right supraclavicular lymph node, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 3/8).

Verified by :

Dr.Toka Medhat



Patient Name : الأستاذ / رباب احمد رمضان	Request No. : 13004179
Patient No. : 49-2020	Request Date : 15/01/2025
Path No. : 175/25	Verification Date : 05/02/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- Her2: Negative (score 0).
 - Ki67: Positive in 20% of tumor cells.
- For confirmatory IPT (GATA3).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghoniem
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

Printing Date : 05/02/2025 12:37:41

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



ImmunohistoChemistry report

Report Number: m383

Female	القوع	رباب احمد رمضان	ام المريض
القاهرة	الخبواج	41 Y	المسن
21/01/2020 12:36	تاريخ امدام لعينة		تاريخ اخذ العينة
02/02/2020 13:01	تاريخ التقرير	مركز اورام دار السلام	طالب التحليل
		مركز اورام دار السلام	عقباً

CLINICAL SAMPLE

Specimen Block

Specimen

ER & HER2 neu (ER&PR: Harvey scoring system - Her2neu: CAP scoring system)

Comment

Specimen

Specimen

The received was treated by: CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody- CONFIRM anti-epidermal growth receptor (PR) (IE2) Rabbit monoclonal primary antibody.

The received was treated by: VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection

Kit, Ventana benchmark GX.

Specimen

1. The received was staining of 30 % of tumour cells

2. The received was staining of 30 % of tumour cells

3. The received was expression: NO membrane staining of tumour cells.

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen (SCORE 6/8)

Specimen (SCORE 5/8)

Specimen: PROTEIN OVER EXPRESSION: NEGATIVE

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Specimen

Lab Director: د/ فريدا ماهر

V

Name : Rabab ahmed ramdan

Branch : Portable

Ref.Physician Prof : cap+c

Date : 29-01-2026 11:57

Medical No. : GRI-15706

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- right mastectomy status.
- Right apical subpleural reticulations likely post therapeutic sequel.
- Linear atelectatic bands are noted
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulæ or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Multilevel vertebral bodies, bilateral iliac bones and sacral sclerotic lesions

Opinion:

- ❖ Right mastectomy status showed no hepatic or pulmonary metastasis.

Name : Rabab ahmed ramdan

Date : 29-01-2026 11:57

Branch : Portable

Medical No. : GRI-15706

Ref.Physician Prof : cap+c

Ins.Company :

❖ **Multilevel vertebral bodies, bilateral iliac bones and sacral sclerotic lesions likely metastatic.**

Dr. Asser Ahmed

Dr. Asser Ahmed

Patient's Name: Rabab Ahmed Ramadan Mohammed
Date: 2026-08-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

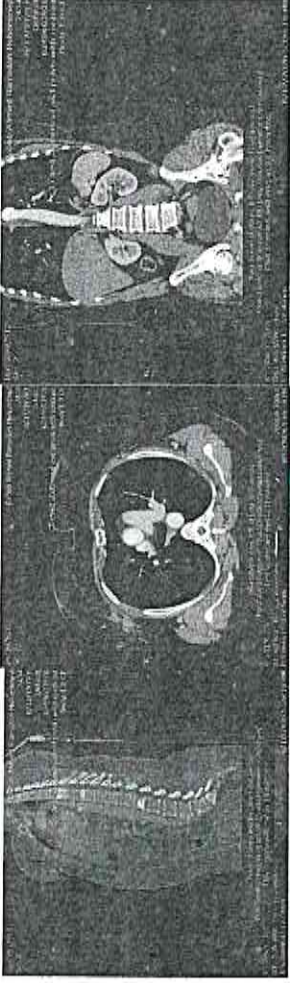
IMPRESSIONS:

A status follow up,compared with the previously study done at 29/01/2026 ,the current study shows:

- Newly-developed two enhancing nodules in the lower outer quadrant of the right breast. For clinical check up with multiple increasing osteosclerotic bony lesions scattering in the scanned spine,pelvic bones on either side and to lesser extent in the sacral bones,likely to be metastatic in nature with a progressive course.
- Hyperinflated both lungs with mild subpleural reticulations in the middle lobe,medial segment of right lower and posterior segment of left lower pulmonary lobes with linear atelectatic band also seen in the left lower pulmonary lobe,likely to be a post therapeutic therapy.
- Fatty infiltrated liver with no focal lesions.

Prof/Hanan El-Ahwal.

Patient's Name: Rabab Ahmed Ramadan Mohammed
Date: 2026-08-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Newly-developed two enhancing nodules are seen in the lower outer quadrant of the right breast with the largest one measures about 36x26mm in dimensions and foci of calcifications scattering inside.
- **Normal left breast.**
- **Benign -looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 12mm.**
- Hyperinflated both lungs.
- Mild subpleural reticulations are seen in the middle lobe,medial segment of right lower and posterior segment of left lower pulmonary lobes with linear atelectatic band seen in the left lower pulmonary lobe.
- **Bilateral basal bronchiectatic changes are also seen.**
- No sizable abnormally-enhancing pulmonary nodules ,other patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Increasing the multiple osteosclerotic bony lesions are seen scattering in the scanned spine,pelvic bones on either side and to lesser extent in the sacral bones.
- Average sized liver,measuring about 14.7 cm, showing a diffuse reduced parenchymal attenuation and a homogenous enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal gland, aorta and IVC.